

KIKADI KAJINGA PATRICIA
AV. CIMBUSHI 4 COMMUNE DE LIMETE 16eme RUE/RESIDENTIEL
Q/MOTEL FIKIN

Transmis Copie Pour information a:



- Monsieur le Directeur General Adjoint du FPC
- Monsieur le Directeur des Ressources Humaines et Services Généraux du FPC
- La Délégation Syndicale du FPC, (TOUS) à Kinshasa/Gombe

A L'ATTENTION DE MADAME

MUTUND KANAM Barbara,

DIRECTEUR GENERAL DUFOND DE PROMOTION CULTURELLE

Concerne : Recours et Contestation de la mesure du licenciement

Par une notification qui serait établie en date du 21 Mai 2026, mais qui a été mise à ma connaissance en date du 5 Juin 2026, vous avez décidé de rompre mon contrat de travail pour motif de maladie, vous trouverez en pièce jointe la notification.

Par la présente, je viens contester cette décision que j'estime discriminatoire et abusive car insinuant une inaptitude non prouvée par les services médicaux ou compétents, inaptitude qui me mettrait dans l'incapacité à exercer une activité au travail.

J'ai signé mon contrat au fond de promotion culturelle en étant drépanocytaire, mon état de santé n'est donc pas nouveau ni méconnu de la direction des ressources humaines, des services de santé de l'entreprise, ni du social, rien ni personne ne m'a pas empêché de contracter ni de répondre à mes obligations en tant que travailleur, compte tenu de l'article 134 de la LOI 2002 portant code du travail, toutes mes hospitalisations aussi ont toujours été signalées, Et, d'ailleurs ne m'ont jamais empêché de répondre à mes obligations en tant que travailleur avec un handicap, Tel que stipulé par l'article 135 de la loi 2002 portant code du travail.

En effet, mes absences ont été dument justifiées par mon Rapport médical délivré par mon médecin traitant du CENTRE DE MEDECINE MIXTE ET D'ANEMIE SS, CMMASS en sigle, c'est pourquoi, selon l'article 57 de la loi 2002 portant code du travail, la suspension du contrat

de travail pour cause de maladie médicalement constatée ne peut donc constituer une faute susceptible de justifier un licenciement unilatéral avec effet immédiat.

Dans le souci de garder mon travail, Et dans mes habitudes de veiller à ma santé, j'ai entamé des démarches de voyage pour me procurer des soins appropriés, démarches pour lesquelles votre apport a été d'un grand soutien puisqu'en date du 30 juillet 2025, pour l'obtention de mon visa pour le Canada afin de me faire soigner, je suis venue en personne vous voir, en vous apportant en cette même date un document de l'immigration canadienne exigeant un model spécifique d'une attestation de service signée par mon employeur, j'y ai joint mon rapport médicale ;Requête que vous avez exaucée en me délivrant une attestation de service , qui n'était pas au model des attestations de services établies par le fond de promotion culturelle, signée par vous et cachetée avec le sceau de l'entreprise, sur un papier avec en tête ayant toutes les informations concernant l'entreprise, et grâce à laquelle j'ai pu obtenir mon visa pour le Canada. Ci joint vous trouverez tous les documents que j'énumère.

Cependant, arrivée au Canada le 21 Octobre 2025, après multiples examens médicaux, il a été constaté encore, qu'une arthroplastie de la hanche droite était nécessaire vue l'état de ma jambe, qui n'était pas méconnu de tout notre personnel de santé du fond de promotion culturelle de la direction générale, ainsi que du service social .

Ici au Canada, Avec des douleurs atroces de la jambe droite, que je supportais depuis plus de 9 ans, J'ai encore dû attendre 6 mois pour rencontrer le spécialiste, J'ai enfin subi une opération chirurgicale par l'orthopédiste en date du 15 Avril 2026 de mon Ostéonécrose aseptique de la tête fémorale droite telle que mentionné dans mon rapport médical, que je vous ai apporté.

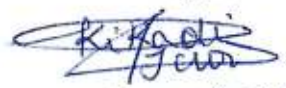
Au Canada, à l'hôpital de North York General de Toronto, ils m'ont donc remplacé les os abimés par une prothèse de hanche, et m'ont rassuré qu'après une bonne rééducation que je fais déjà d'ailleurs auprès du physiothérapeute, et avec un bon moment de repos, je pourrais reprendre avec mes activités, vous êtes sans ignorer qu'une opération chirurgicale nécessite un temps considérable de récupération.

Au vu de tous ces éléments, je vous prie de bien vouloir revoir votre décision et procéder à l'annulation de cette mesure de licenciement et de me rétablir dans mes droits...

Dans l'attente d'une suite favorable, je vous prie d'agréer. Madame Le Directeur General du FPC, l'expression de mes salutations distinguées.

KIKADI KAJINGA PATRICIA,

Votre agent


Le 17 juin 2026

PIECES JOINTES :

- Notification du licenciement par le DG du FPC
- Attestation de service signée par la DG du FPC, selon le modèle de l'immigration canadienne
- Mon Rapport Médical du CMMASS par mon médecin soignant
- Lettre de l'immigration exigeant un document avec une description signée par mon employeur avec le sceau de l'entreprise en qualité de chef de bureau adjoint à la direction des ressources humaines et services généraux
- Tous mes documents de l'hôpital au Canada : Fiche de consultation et bon de laboratoire par mon médecin de famille ; Lettre de sortie après hospitalisations et diverses recommandations médicales au North York General Hospital de Toronto, les prescriptions et rapport de l'hôpital, ainsi que les images ou photos de mes différentes hospitalisations et de ma plaie après la chirurgie.



Direction Générale

N° 317 /FPC/DG/SEC-DG/GPN/BN/2026

Transmis copie pour information à :

- Monsieur le Directeur Général Adjoint du FPC;
 - Monsieur le Directeur Général Adjoint du FPC ;
 - Monsieur le Directeur des Ressources Humaines et Services Généraux du FPC ;
 - La Délégation Syndicale du FPC.
- (Tous) à Kinshasa/Gombe.

✓ A Madame KIKADI KAJINGA Patricia
C/° Direction des Ressources Humaines et Services
Généraux du FPC
à Kinshasa/Gombe

Concerne : Votre licenciement avec préavis.

Madame,

Conformément aux résultats de la Commission Chargée de l'actualisation et de la fiabilisation des listes des effectifs du personnel, établie à la suite du contrôle des dossiers administratifs, qui s'est réunie du lundi 23 au vendredi 27 mars 2026 afin d'identifier notamment le personnel actif ainsi que les différents cas d'agents passifs, et aux différents rapports des coordinations provinciales disponibles.

Après analyse, votre dossier révèle des longues absences de plusieurs mois dues principalement à votre situation de maladie prolongée ayant affectée votre capacité physique à reprendre normalement le service. Ce qui revient à dire que cette inaptitude vous met dans l'incapacité à exercer une activité au travail.

Pour ce motif, et conformément à l'article 118 du Statut du Personnel du Fonds de Promotion Culturelle, j'ai décidé la résiliation de votre contrat de travail **avec préavis**, prenant effet **dès réception de la présente**.

Le Directeur en charge des Ressources Humaines et Services Généraux, qui me lit en copie, est chargé de procéder à la clôture de votre compte

patriotiques.

Veuillez agréer, **Madame**, l'expression de mes sentiments

MUTUND KANAM Barbara

Directeur Général



REPUBLIQUE DEMOCRATIQUE DU CONGO
MINISTÈRE DE LA RECHERCHE SCIENTIFIQUE ET INNOVATION TECHNOLOGIQUE
INSTITUT DE RECHERCHE EN SCIENCES DE LA SANTÉ
CENTRE DE MÉDECINE MIXTE ET D'ANÉMIE SS

CMMASS

RAPPORT MEDICAL N° 050.../2025

Nous soussignés, Dr. TONGU NGBO Excellence, Dr. KAMBANSI WABA Trésor, Dr. KALENGA MIKANDA, régulièrement inscrits au tableau de l'Ordre des Médecins Congolais, certifions avoir examinés et suivis au Centre de Médecine Mixte et d'Anémie SS (CMMASS) Mademoiselle KIKADI KAJINGA Patricia pour un syndrome drépanocytaire majeur.

Mademoiselle KIKADI KAJINGA Patricia, née à LUBUMBASHI le 19 Juin 1987, de Nationalité Congolaise, résidant au n°4 de l'avenue CIMBUSHI, 16ème Rue LIMETE, Commune de LIMETE, dans la Ville de Kinshasa en République Démocratique du Congo.

MODE DE VIE

- Vit dans le domicile familial avec sa mère et ses frères et sœurs.
- Agent au fond de promotion culturelle (FPC) depuis 2017
- Pas d'animaux de compagnie au domicile
- N'a jamais fumé, ne prend pas d'alcool.

ANTECEDENTS

Antécédents familiaux

- Père décédé
- Mère en vie et en bonne santé
- 6^{ème} dans une fratrie de 13 enfants dont elle est la seule drépanocytaire malade.

Antécédents médicaux

- Intolérance non connue
- Allergie non connue
- Hépatite B : séronégatif
- Sérologie VIH : séronégatif

- Drépanocytaire homozygote, diagnostic à l'électrophorèse de l'hémoglobine à l'âge de 2 ans, suivie au centre de médecine mixte et d'anémie SS.
- Hémoglobine de base 7 g/dl
- Nombre de transfusion : 7 transfusions dont la dernière de 900 ml de culot globulaire remonte au mois de janvier 2015 dans notre institution médicale
- Crise vaso-occlusive ostéo-articulaire sévère 1 à 3 fois / an avec des nombreuses hospitalisations depuis sa naissance.
- Ostéonécrose aseptique de la tête fémorale droite
- Multiples infections au plasmodium et traitées par l'artesunate.

VACCINS SPECIFIQUES ADMINISTRES:

Pneumovax-23
Vaxigrip
Typhim-Vi
EngerixB

SITUATION SANITAIRE

Bilans réalisés

FEVRIER 2025	
Hb	7,6 g/dl
Plaquettes	173 000/ μ l
FL :	
PNN	55,0 %
Lymp	33,0%
Mono	7,0 %
Eosinophile	5,0 %
Basophile	0 %
Réticulocytes	1,5 %
GB	10 500 / μ l
GR	4040000/ μ l
GE	Négative
GLYCEMIE A JEUN	79 mg%
Ac ANTI HBs	28,46 mUI. /ml
Hépatite C	Négatif
VS	5 mm/ 1ere HEURE
Sérologie à VIH	Négative

Cliniquement (Rdv médical du mois de JUIN 2025)

Elle avait comme plaintes :

- Légère douleur ostéo-articulaires de la hanche droite.
- L'examen clinique du jour était dominé par :
 - un discret ictère conjonctival
 - Mobilisation passive et active douloureuse de la hanche droite.
 - Raccourcissement du membre inférieur droit d'environ 3 centimètres par rapport à celui de gauche.
- La dernière radiographie du bassin fait état dans ses conclusions d'une ostéonécrose aseptique de la tête fémorale droite au stade III de Arlet et Fica.

OPINION SUR LE CAS

En dehors de l'ostéonécrose aseptique de la tête fémorale droite comme complication de la maladie que présente Mademoiselle **KIKADI KAJINGA Patricia**, les contrôles médicaux réguliers associés à un mode de vie sain font qu'elle soit une patiente cliniquement stable.

Ce rapport est établi en toute conscience et responsabilité pour servir à toute fin utile.

Fait à Kinshasa le 11/07/2025

Médecins Traitants

Dr. TONGU NGBO
Excellence

Cnom 2770

Dr. KAMBANSI WABA



Dr. KALENGA MUKINDA
Baron

Cnom 13738



Immigration, Réfugiés
et Citoyenneté Canada

Immigration, Refugees
and Citizenship Canada

Date : 23 juillet 2025

ICU : 11-4870-0596

Demande : E003335210

Patricia Kajinga Kikadi

Courriel : kajingakikadi@gmail.com

Madame, Monsieur,

La présente concerne votre demande de résidence permanente au Canada dans la catégorie des travailleurs qualifiés du Québec (Entrée express).

Nous avons commencé à traiter votre demande. Pour prendre une décision, nous avons besoin des renseignements/documents suivants :

- Veuillez fournir une description de poste signée par l'employeur avec le sceau de l'entreprise de votre emploi en qualité de Chef de bureau adjoint à la direction des ressources humaines et services généraux chez le Fonds de Promotion Culturelle.

À noter :

Les renseignements/documents ci-dessus doivent être reçus dans un délai de **15 jours calendrier** à compter de la réception de la présente lettre. Si vous ne respectez pas ce délai, votre demande sera évaluée en se basant sur les renseignements que vous avez fournis dans votre demande, et celle-ci pourrait être rejetée. Veuillez téléverser ces documents dans votre compte MonCIC.

Si vous n'êtes pas en mesure de fournir les documents ou renseignements demandés, vous devez nous aviser par écrit en nous indiquant pour quelle(s) raison(s) vous n'êtes pas en mesure de nous fournir les documents ou renseignements demandés. Vous devez nous soumettre cette explication par voie électronique du portail de MonCIC en remplacement du document demandé.

Si vous rencontrez des difficultés lors du téléversement de documents dans votre compte MonCIC, vous pouvez utiliser le [formulaire Web d'IRCC](#).

Tous les documents qui sont dans une langue autre que le français ou l'anglais doivent être accompagnés d'une traduction conforme vers le français ou l'anglais effectuée par un traducteur certifié. Lorsque les documents originaux sont demandés, vous devez télécharger les documents originaux à MonCIC. Nous n'acceptons pas les copies signées par un notaire.

*****IMPORTANT***** Vous devez informer notre bureau lorsqu'une des situations suivantes s'applique à vous :

- Mariage, union de conjoint de fait, divorce ou séparation légale;
- Décès du demandeur principal ou d'un membre de la famille qui accompagne le demandeur principal ou du parrain;
- La naissance d'un enfant ou l'adoption d'un enfant. Ceci inclut la naissance d'un enfant d'un de vos enfants à charge;

Immigration, Refugees and Citizenship Canada | Immigration, Réfugiés et Citoyenneté Canada
Government of Canada | Gouvernement du Canada
www.canada.ca/immigration

- Si vous, le demandeur principal, ou un membre de votre famille est accusé ou reconnu coupable d'un crime dans un pays quelconque.

Veuillez agréer, Madame, Monsieur, mes sincères salutations.

Immigration, Refugees and Citizenship Canada | Immigration, Réfugiés et Citoyenneté Canada
Government of Canada | Gouvernement du Canada

PRIVATE AND CONFIDENTIAL

If you are not the intended recipient of this document, please contact NYGH immediately at 416-756-6209



**NORTH
YORK
GENERAL**

4001 Leslie Street
Toronto, Ontario M2K 1E1

Patient : KIKADI, PATRICIA KAJINGA
DOB/Age/Sex : 6/19/1987 38 years Female
Visit Type : Emergency
Family MD : Ibiyooye, Ademola Babatunde
Attend MD :

MRN: 2519410
HCN: 4557552181
FIN: 0009368402
Admit Date: 11/10/2025
Disch. Date:

Emergency Documentation

Document Name: Emergency Physician Assessment Document Status: Auth (Verified)

Name: KIKADI, PATRICIA KAJINGA
Address:
2911 BAYVIEW AVENUE
A210
NORTH YORK, ONTARIO M2K1E8
Sex: Female
Date of Birth: 06/19/1987
Phone: 4376635550
Emergency Contact: KIKADI, DEDE
Age: 38 Years

MRN: 2519410
Location: North York General Hospital
Health Card Number: 4557552181 KJ
Family Physician: Ibiyooye, Ademola Babatunde

Visit Information

Admit Date/Time: 10-NOV-2025 15:56
Encounter Type: Emergency

Time

11/10/2025 16:06

Allergies

tramadol Dizziness, Vomiting

Chief Complaint

Joint pain

Past Medical History

Sickle cell disease

Medications

Morphine 5 mg as needed
ASA 81 mg as needed with morphine
Hydromorphone 1 mg as needed

History of Present Illness

History and exam completed with french translation.

38-year-old female with a history of sickle cell disease presenting with acute onset joint pain in her lower back bilaterally, bilateral elbows and knees since 9 AM this morning. States that her pain is currently an 8 out of 10 in severity, has been stable following initial dose of the 100 mcg of fentanyl SL in the ED.

Patient was recently discharged for sickle cell vaso-occlusive crisis involving diffuse joint pain on Nov 9th from this . She received 2 doses of SL fentanyl 100mcg, then 3mg of hydromorphone (2mg + 1mg) SC following which she had pain resolution and was able to go home. She states that since going home she has had increased pain to her lower back, elbows, knees. She has had good p.o. intake at home and has been hydrating. She took 5 mg of morphine as well as 5 mg of hydromorphone without any benefit to her pain leading to repeat ED presentation. She denies any fevers at home, chest pain, or shortness of breath.

In regards to her back pain, she denies any peripheral weakness, numbness, perianal anesthesia, difficulty urinating, or loss of control of bowel or bladder. She again denies any fevers at home. She states that the pain is bilateral across her lower back with no radiation.

She has recently moved to Canada and has a family physician. She has been referred through her family physician to a hematologist for management of her sickle cell but has not currently got an appointment scheduled, currently has not initiated hydroxyurea. She has a history of joint necrosis to her right hip for which her family physician has referred her to orthopedic surgery for further assessment, has not currently heard back. She denies any prior history of ACS. No history of other sickle cell complications including osteomyelitis.

She has been able to ambulate.

RRID: 67475623

PRIVATE AND CONFIDENTIAL

If you are not the intended recipient of this document, please contact NYGH immediately at 416-756-6209



Patient :	KIKADI, PATRICIA KAJINGA	MRN:	2519410
DOB/Age/Sex :	6/19/1987 38 years Female	HCN:	4557552181
Visit Type :	Emergency	FIN:	0009368402
Family MD :	Ibiyooye, Ademola Babatunde	Admit Date:	11/10/2025
Attend MD :		Disch. Date:	

Emergency Documentation

Physical Exam

Vitals & Measurements

ED Triage: **Temp:** 36.6 DegC (Temporal) **HR:** 101 bpm, **RR:** 18 **BP:** 111/48 **SpO2:** 99% (10-NOV-2025 16:40) | Mode of Arrival: Walk In

General: Patient is alert, no acute distress

CVS: S1/S2, no murmurs, peripheral pulses 2+

Resp: Breath sounds clear to auscultation bilaterally no crackles or wheeze, no increased work of breathing on assessment

Back exam: No midline tenderness. No obvious deformity or areas of fluctuance to palpation. Power 5 out of 5 throughout lower limb bilaterally, no clonus. Sensation intact to periphery. Reflexes 2+2 patellar and Achilles reflex. Gait normal, no foot drop. Peripheral pulses 2+.

Elbow exam: Warmth to palpation of both elbows, no significant swelling. No tenderness to palpation. Full ROM intact without significant pain. Radial pulses 2+, power 5 out of 5 to periphery. Cap refill less than 2 seconds.

Knee exams: Warm to palpation of both knees. Nontender to palpation. Full ROM intact without significant pain.

Hip exam: No significant erythema or swelling. Mild tenderness to anterior palpation. Limited ROM with hip flexion, patient states this is her baseline.

Course in ED

100 mcg fentanyl received 1539

100 mcg fentanyl ordered 1715 (200mcg total 1715)

HM 1mg SC q15 minutes x3 ordered (3mg total)

Ondansetron and cetirizine ordered as needed for nausea or pruritus as opioid side effects respectively

Spoke with hematology given recurrent presentation from Nov 9th for VOC pain, unfortunately hematology does not follow individuals for sickle cell disease at NYGH, do not recommend starting hydroxyurea in ED

Reassessment 18:34 - Patient has received 200mcg fentanyl SL and 2mg HM SC at present, reports pain is 7/10. Denies any CP/SOB development.

Reassessed 19:30 - Patient has received 200mcg fentanyl SL and 3mg HM SL at present, pain 5/10, comfortable for discharge with PO meds. Endorses that she is unsure if referral has been sent to hematologist by family physician, will refer to UHN hematology for follow up assessment. Will give HM 2 mg p.o. prior to discharge.

Labs

Labs

Glucose Random: 5.9 mmol/L (11/10/25 20:10:00)

Creatinine: 36 UMOL/L Low (11/10/25 20:10:00)

Total Bilirubin: 83 UMOL/L High (11/10/25 20:10:00)

ALP: 201 U/L High (11/10/25 20:10:00)

ALT: 28 U/L (11/10/25 20:10:00)

LDH: 607 U/L High (11/10/25 20:10:00)

Quant. BhCG: <4 (11/10/25 20:10:00)

Lactate: 3.2 mmol/L High (11/10/25 17:13:00)

Lipase Level: 39 U/L (11/10/25 20:10:00)

Sodium: 138 mmol/L (11/10/25 20:10:00)

Potassium: 4.1 mmol/L (11/10/25 20:10:00)

Chloride: 106 mmol/L (11/10/25 20:10:00)

Total CO2: 20 mmol/L Low (11/10/25 20:10:00)

Anion Gap: 12 mmol/L (11/10/25 20:10:00)

pH Venous: 7.41 (11/10/25 20:10:00)

pCO2 Venous: 35 MM HG Low (11/10/25 20:10:00)

Bicarbonate Venous: 22 mmol/L Low (11/10/25 20:10:00)

Base Excess Venous: -2 mmol/L (11/10/25 20:10:00)

Retic Count: 317 10E9/L High (11/10/25 20:10:00)

RRID: 67475623

PRIVATE AND CONFIDENTIAL

If you are not the intended recipient of this document, please contact NYGH immediately at 416-756-6209



**NORTH
YORK
GENERAL**

4001 Leslie Street
Toronto, Ontario M2K 1E1

Patient : KIKADI, PATRICIA KAJINGA
DOB/Age/Sex : 6/19/1987 38 years Female
Visit Type : Emergency
Family MD : Ibiyooye, Ademola Babatunde
Attend MD :

MRN: 2519410
HCN: 4557552181
FIN: 0009368402
Admit Date: 11/10/2025
Disch. Date:

Emergency Documentation

CBC

HGB: 76 g/L Low (11/10/25 20:10:00)
LKC: 9.8 10E9/L (11/10/25 20:10:00)
Platelets: 357 10E9/L (11/10/25 20:10:00)
MCV: 75 fL Low (11/10/25 20:10:00)
RDW-CV: 31 % High (11/10/25 20:10:00)
HCT: 0.212 L/L Low (11/10/25 20:10:00)
ERC: 2.83 10E12/L Low (11/10/25 20:10:00)
MCH: 27.1 pg Low (11/10/25 20:10:00)
MCHC: 360 g/L (11/10/25 20:10:00)

Impression/Assessment/Plan

38F with sickle cell disease presenting with joint pain, presentation in line with VOC with no features on history or exam concerning for acute bone ischemia or osteomyelitis. No chest pain or SOB. Patient given a total of 200 mcg of fentanyl SL (100mcg x 2) as well as HM 3mg SC (1mg x 3) and HM 2mg po x1 in the ED with reduction of her pain to 4-5/10 on reassessment, comfortable for discharge home. Patient denies any drowsiness, nausea, or pruritus on reassessment. No chest pain or SOB, no concern for acute chest syndrome with normal O2 sats. Prescription given for hydromorphone oral doses at home for further pain control, prescribed 2 mg every 4 hours as a standing dose for pain episodes with breakthrough dose of hydromorphone 1 mg p.o. every hour. Patient referred to UHN hematology for follow-up assessment and further management of her sickle cell disease. Hgb electrophoresis sent.

ED Diagnosis

Vaso-occlusive crisis

Patient Discharge Instructions

We prescribed you hydromorphone to be taken orally for further management of your pain crisis, you can take the standing dose of 2 mg every 4 hours as needed for your pain as well as an additional dose of 1 mg every hour as needed for breakthrough pain. Please return to emergency department if you experience any fevers, worsening joint pain with difficulty moving any of your joints, chest pain, or shortness of breath, or if you are otherwise concerned. Otherwise we have referred you to the UHN hematology clinic for further assessment and management of your sickle cell disease.

Prescriptions

Hydromorphone 2 mg every 4 hours as needed
Hydromorphone 1 mg every 1 hour as needed

Follow Up

Hematology outpatient clinic

Electronically Signed By: Murphy, Lara Elizabeth Resident
On: 10-Nov-2025 21:12

Copies To:

Wu, Adrian Mitchell MD, FRCPC, DRCPC

The family physician on record will automatically receive a copy of this report.

PLEASE NOTE: Elements of this report were generated using electronic technology including speech recognition.

RRID: 67475623

Discharge Summary

KIKADI, Ms. PATRICIA KAJINGA - 2519410

Result type: Discharge Summary
Service date: 2025-Dec-03 11:26 EST
Result status: Modified
Result title: Discharge Summary
Performed by: Zhang, Zhi Rui Resident on 2025-Dec-03 11:27 EST
Verified by: Zhang, Zhi Rui Resident on 2025-Dec-03 11:27 EST
Encounter info: 0009384311, NYGH, Inpatient, 2025-Nov-22 -

Document Has Been Revised

Name: KIKADI, PATRICIA KAJINGA
Address:
2911 BAYVIEW AVENUE
A210
NORTH YORK, ONTARIO M2K1E8
Sex: Female
Date of Birth: 06/19/1987
Phone: 4376635550
Emergency Contact: KIKADI, DEDE
Age: 38 Years

MRN: 2519410
Location: North York General Hospital
Health Card Number: 4557552181 KJ
Family Physician: Ibiyooye, Ademola
Babatunde

Visit Information

Admit Date/Time: 22-NOV-2025 15:41
Encounter Type: Inpatient
Attending Provider: Bader, Russell Todd
MD, FRCPC

Hospital Course

Ms. Kikadi is a 38-year-old female with history of sickle cell disease (compound heterozygote S beta thalassemia) who recently immigrated from Congo to Canada in October. She has presented with recurrent episodes of vaso-occlusive crisis over the last month and was admitted again on November 22. Her major complaints of pain were in her bilateral elbows and knees. Cold weather was identified as a potential trigger for her pain. She was started on hydromorphone subcutaneously with a gradual taper. She will be discharged with hydromorphone p.o. 2 mg every 4 hours as needed.

Chest x-ray was negative for any intrathoracic disease. She intermittently required minimal oxygen for mild desaturations but overall did not have any signs or symptoms consistent with acute chest syndrome.

She previously had investigation for right hip AVN with her family physician and was awaiting an appointment with orthopedics. We did repeat her hip x-rays during her admission which showed very severe right hip AVN and possible early left AVN. We connected her with Dr. Mehdiian from orthopedic surgery who has placed her on the surgical list and will follow her as an outpatient.

She was previously referred to the UHN RBC clinic however there is a long wait list. We have referred her to the Scarborough sickle cell clinic on discharge.

Her hemoglobin remained in the 60-70 range during hospitalization which she stated is her baseline. She does not become symptomatic until her hemoglobin is in 50 and she declined transfusions.

Selected Notes & Imaging Results

(11/27/2025 11:19 EST Pelvis/ Hip Bilateral)

The pelvic ring is intact. Patchy areas of sclerosis involving both iliac wings suspected related to underlying history of sickle cell disease.

Severe chronic changes right hip AVN of the superior lateral femoral head with flattening and remodeling of both the femoral head and acetabular roof.

Allergies

tramadol Itch, Dizziness, Vomiting

Last Charted Vitals & Measurements (Last 24 Hrs)

T: 36.6 °C (Oral) TMIN: 36.6 °C (Oral)
TMAX: 37.0 °C (Oral)
HR: 65 (Peripheral) RR: 18 BP: 120/63
SpO2: 95%

Selected Lab Results (Last 24 Hrs)

No qualifying data available.

Additional Pertinent Lab Results

Test Name	Test Result	Date/Time
Creatinine	± 38 UMOL/L	12/01/2025 06:45 EST
Total Bilirubin	± 57 UMOL/L	12/01/2025 06:45 EST
Direct Bilirubin	± 10 UMOL/L	12/01/2025 06:45 EST
LDH	± 832 U/L	12/01/2025 06:45 EST
Sodium	137 mmol/L	12/01/2025 06:45 EST
Potassium	± 4.8 mmol/L	12/01/2025 06:45 EST
Chloride	107 mmol/L	12/01/2025 06:45 EST
Total CO2	± 17 mmol/L	12/01/2025 06:45 EST
Anion Gap	12 mmol/L	

Printed by: Zhang, Zhi Rui Resident
Printed on: 2025-Dec-03 11:31 EST

Discharge Summary

KIKADI, Ms. PATRICIA KAJINGA - 2519410

Result type: Discharge Summary
Service date: 2025-Dec-03 11:26 EST
Result status: Modified
Result title: Discharge Summary
Performed by: Zhang, Zhi Rui Resident on 2025-Dec-03 11:27 EST
Verified by: Zhang, Zhi Rui Resident on 2025-Dec-03 11:27 EST
Encounter info: 0009384311, NYGH, Inpatient, 2025-Nov-22 -

Left hip joint space well preserved however patchy areas of sclerosis are evident within the left femoral head and neck. Potential 2.1 area of mild lucency with sclerotic margin involving the superior weightbearing surface of the left femoral head suspicious for AVN with questionable thin crescentic lucency best seen on the lateral study without step deformity recommended for MR assessment as clinically warranted [1] (11/09/2025 12:19 EST)

Chest PA / Lateral)
FINDINGS:

No confluent airspace opacities. Cardiomedastinal structures intact. No pleural fluid.

IMPRESSION:

No active findings. [2]

Goals of Care

Resuscitation Status: 11/22/25 20:37:00 EST, Pt/SDM Resus Decision: FULL Resuscitation, Resus Decision From: Patient

Test Name	Test Result	Date/Time
		12/01/2025 06:45 EST
Retic Count	↑ 274 10E9/L	12/01/2025 08:20 EST
HGB	◆ 67 g/L	12/01/2025 08:20 EST
LKC	9.3 10E9/L	12/01/2025 08:20 EST
Platelets	↑ 417 10E9/L	12/01/2025 08:20 EST
MCV	↓ 82 fL	12/01/2025 08:20 EST
RDW-CV	↑ 24 %	12/01/2025 08:20 EST
HCT	↑ 0.185 L/L	12/01/2025 08:20 EST
ERC	↑ 2.27 10E12/L	12/01/2025 08:20 EST
MCH	29.7 pg	12/01/2025 08:20 EST
MCHC	↑ 364 g/L	12/01/2025 08:20 EST
Neutrophil	NA	11/28/2025 10:28 EST
Lymphocyte	NA	11/28/2025 10:28 EST
Monocyte	NA	11/28/2025 10:28 EST
Eosinophil	NA	11/28/2025 10:28 EST
Basophil	NA	11/28/2025 10:28 EST
Neutrophil (Manual dif f)	3.5 10E9/L	12/01/2025 08:20 EST
Lymphocyte (Manual dif f)	↑ 4.8 10E9/L	12/01/2025 08:20 EST
Monocyte (Manual dif f)	0.6 10E9/L	12/01/2025 08:20 EST

Printed by: Zhang, Zhi Rui Resident
Printed on: 2025-Dec-03 11:31 EST

Discharge Summary

KIKADI, Ms. PATRICIA KAJINGA - 2519410

Result type: Discharge Summary
Service date: 2025-Dec-03 11:26 EST
Result status: Modified
Result title: Discharge Summary
Performed by: Zhang, Zhi Rui Resident on 2025-Dec-03 11:27 EST
Verified by: Zhang, Zhi Rui Resident on 2025-Dec-03 11:27 EST
Encounter info: 0009384311, NYGH, Inpatient, 2025-Nov-22 -

Test Name	Test Result	Date/Time
Eosinophil (Manual diff)	↑ 0.3 10E9/L	12/01/2025 08:20 EST
Basophil (Manual diff)	0.0 10E9/L	12/01/2025 08:20 EST
Myelocyte	0.1 10E9/L	12/01/2025 08:20 EST
Nerc/100 LK C	4 /100 LKCs	12/01/2025 08:20 EST
Microcytosis	1+	11/25/2025 06:36 EST
Target Cell	1+	12/01/2025 08:20 EST
Elliptocyte	1+	11/25/2025 06:36 EST
Polychromasia	2+	12/01/2025 08:20 EST
Sickle Cells	3+	12/01/2025 08:20 EST
Spherocytes	Not Significant	12/01/2025 08:20 EST
How'l Jol Body	1+	11/25/2025 06:36 EST
PLT, EST	Increased	12/01/2025 08:20 EST
PLT, Giant	Noted	11/23/2025 09:00 EST
Large Platelets	Noted	12/01/2025 08:20 EST

Medications

	What	How Much	When	Instructions
Unchanged	HYDROmorphine	2 Milligram Oral	Every 4 hours as needed for Pain	Printed Prescription
Unchanged	Non-Formulary (Vicks Rub)	1 application Topical	Daily as needed for Pain	

	What	How Much	When	Comments
Stop Taking	acetylsalicylic acid (acetylsalicylic acid enteric coated)	81 Milligram Oral	Daily as needed for Pain	

INFORMATION FOR PATIENT:

Discharge Summary

KIKADI, Ms. PATRICIA KAJINGA - 2519410

Result type: Discharge Summary
Service date: 2025-Dec-03 11:26 EST
Result status: Modified
Result title: Discharge Summary
Performed by: Zhang, Zhi Rui Resident on 2025-Dec-03 11:27 EST
Verified by: Zhang, Zhi Rui Resident on 2025-Dec-03 11:27 EST
Encounter info: 0009384311, NYGH, Inpatient, 2025-Nov-22 -

Patient Discharge Instructions

1. On vous a prescrit de l'hydromorphone a raison de 2 mg par voie orale toutes les 4 heures, au besoin. Si vous avez besoin de comprimés supplémentaires, veuillez contacter votre médecin de famille.
2. Vous pouvez vous procurer du Tylenol, de l'ibuprofène et de l'acide folique sans ordonnance. Prenez le Tylenol et l'ibuprofène conformément aux instructions sur l'emballage. Prenez 1 mg d'acide folique par jour.
3. Vous avez consulté le Dr Mehdian, chirurgien orthopédiste, pour votre ostéonécrose de la hanche droite. Veuillez appeler pour fixer un rendez-vous.
4. Vous avez été orienté(e) vers la clinique de drepanocytose de l'hôpital de Scarborough. Ils communiqueront avec vous pour fixer un rendez-vous. (Centenary hospital, 2867 Ellesmere Road, 416-281-7442)

Follow Up Instructions

With	When	Contact Information
General - Total Joint Assessment Clinic	Within 4 to 6 Weeks	4001 Leslie Street Toronto, ONTARIO M2K 1E1 (416) 756-6675
Additional Instructions: Prenez rendez-vous avec le Dr Mehdian, spécialiste en orthopédie. Veuillez appeler pour fixer un rendez-vous.		

- [1] Pelvis/ Hip Bilateral; LaMere, Elizabeth 11/27/2025 11:19 EST
[2] Chest PA / Lateral; Jacobs, Irving 11/09/2025 12:19 EST

Signature Line

Electronically Signed By: Zhang, Zhi Rui Resident
On: 03-Dec-2025 11:27

Electronically Signed By: Zhang, Zhi Rui Resident
On: 03-Dec-2025 11:30

Copies To:

Bader, Russell Todd MD, FRCPC
Mehdian, Hossein MD, FRCSC
Ibiyooye, Ademola Babatunde

The family physician on record will automatically receive a copy of this report.

PLEASE NOTE: Elements of this report were generated using electronic technology including speech recognition.

Completed Action List:

- * Perform by Zhang, Zhi Rui Resident on 2025-Dec-03 11:27 EST
- * Sign by Zhang, Zhi Rui Resident on 2025-Dec-03 11:27 EST
- * VERIFY by Zhang, Zhi Rui Resident on 2025-Dec-03 11:27 EST

Printed by: Zhang, Zhi Rui Resident
Printed on: 2025-Dec-03 11:31 EST

Discharge Summary

KIKADI, Ms. PATRICIA KAJINGA - 2519410

Result type: Discharge Summary
Service date: 2025-Dec-03 11:26 EST
Result status: Modified
Result title: Discharge Summary
Performed by: Zhang, Zhi Rui Resident on 2025-Dec-03 11:27 EST
Verified by: Zhang, Zhi Rui Resident on 2025-Dec-03 11:27 EST
Encounter info: 0009384311, NYGH, Inpatient, 2025-Nov-22 -

* Modify by Zhang, Zhi Rui Resident on 2025-Dec-03 11:30 EST

* Sign by Zhang, Zhi Rui Resident on 2025-Dec-03 11:30 EST

Discharge Summary

KIKADI, Ms. PATRICIA KAJINGA - 2519410

Result type: Discharge Summary
Service date: 2026-Jan-27 13:38 EST
Result status: Auth (Verified)
Result title: Discharge Summary
Performed by: Mollanji, Roland MD, CCFP, FCFP on 2026-Jan-27 13:39 EST
Verified by: Mollanji, Roland MD, CCFP, FCFP on 2026-Jan-27 13:39 EST
Encounter info: 0009412492, NYGH, Inpatient, 2025-Dec-14 -

Name: KIKADI, PATRICIA KAJINGA
Address:
2911 BAYVIEW AVENUE
A210
NORTH YORK, ONTARIO M2K1E8
Sex: Female
Date of Birth: 06/19/1987
Phone: 4376635550
Emergency Contact: KIKADI, DEDE
Age: 38 Years

MRN: 2519410
Location: North York General Hospital
Health Card Number: 4557552181 KJ
Family Physician: Ibiyooye, Ademola
Babatunde

Visit Information

Admit Date/Time: 14-DEC-2025 04:52
Encounter Type: Inpatient
Attending Provider: Mollanji, Roland MD,
CCFP, FCFP

Primary Discharge Diagnosis

Vaso occlusive crisis

Hospital Course

ID: 38F, hx sickle cell, readmitted with vaso occlusive crisis one day after discharge for the same

PMedhx:

1. Sickle cell disease (S/Beta thalassemia)
 - Recently referred to the UHN RBC clinic but has not yet had an appointment
 - AVN of right hip, referred by family physician to orthopedic surgery but still waiting for appointment
 - Previous documentation reports hemoglobin SS disease, however hemoglobin electrophoresis shows compound heterozygote of hemoglobin S and beta thalassemia ay NYGH Nov 10 2025

HPI:

Ms Kikadi presented to North York General Hospital on December 14, one day after being discharged for a vaso-occlusive crisis secondary to sickle cell disease.

She was admitted to North York General Hospital December 6-13 with vaso-occlusive crisis. She required hydromorphone 2 mg SQ every 2 hours, with 1 mg as needed. She requested discharge directly home without weaning. The cold triggered her symptoms, and she presented to hospital the following day. Her hemoglobin and bilirubin were found to be at her baseline's, Hb 69, and T-bili 49 respectively.

She has had multiple admissions for same since she came to Canada in October 2025 from Congo. She was readmitted to hospital for management of vaso occlusive crisis.

She has had a prolonged hospitalization the main issue has been pain control, She has been quite reluctant to be switched from parenteral to oral hydromorphone, She has been complaining of pain, She seems to be quite intolerant to cold. She started on hydromorphone CR, and started on Celebrex. In the last week her pain has gradually improved,

Allergies

tramadol Itch, Dizziness, Vomiting

Last Charted Vitals & Measurements (Last 24 Hrs)

T: 37.0 °C (Axillary) **TMIN:** 37.0 °
C (Axillary) **TMAX:** 37.1 °C (Oral)
HR: 81 (Peripheral) **RR:** 17 **BP:** 102/60
SpO2: 98%

Selected Lab Results (Last 24 Hrs)

Cardiac/Endocrine
TSH: 3.7 MU/L (01/27/26 09:28:00)

Discharge Summary

KIKADI, Ms. PATRICIA KAJINGA - 2519410

Result type: Discharge Summary
 Service date: 2026-Jan-27 13:38 EST
 Result status: Auth (Verified)
 Result title: Discharge Summary
 Performed by: Mollanji, Roland MD, CCFP, FCFP on 2026-Jan-27 13:39 EST
 Verified by: Mollanji, Roland MD, CCFP, FCFP on 2026-Jan-27 13:39 EST
 Encounter info: 0009412492, NYGH, Inpatient, 2025-Dec-14 -

She is willing to go and try at home today, however I am afraid that she may rebound as she may not tolerate the cold weather,
 Seen by social worker, provided support, referred to TAMPI pain clinic, NYGH mental health program for mindfulness for her medical condition, and referred to North York, community Settlement worker. Also was provided with info re social/mental health support related to sickle cell in the community.
 Awaiting to get appointment for RBC clinic and time for OR re AVC with Dr. Mehdian

HK
 DVTp dalt
 Full code
 Dispo: home, stable condition

Medications

	What	How Much	When	Instructions
<i>New</i>	celecoxib (CeleBREX)	200 Milligram Oral	Twice daily with meals	Printed Prescription
<i>New</i>	diclofenac topical (VOLTAREN 1.16% EMULGEL)	1 application Topical	3 times daily	Printed Prescription
<i>New</i>	senna (Senokot)	17.2 Milligram Oral	At bedtime	Printed Prescription
<i>Changed</i>	HYDROmorphine	4 Milligram Oral	Every 4 hours as needed for Pain	Printed Prescription
<i>Changed</i>	HYDROmorphine (HYDROmorphine CR)	12 Milligram Oral	Every 8 hours	Printed Prescription
<i>Changed</i>	acetaminophen (acetaminophen extra strength)	1,000 Milligram Oral	Every 8 hours as needed for Pain	Printed Prescription
<i>Changed</i>	hydroxyurea	500 Milligram Oral	Daily	
<i>Unchanged</i>	folic acid	5 Milligram Oral	Daily	

	What	How Much	When	Comments
<i>Stop Taking</i>	acetylsalicylic acid (Aspirin enteric coated)	81 Milligram Oral	As needed for Pain	

INFORMATION FOR PATIENT:**Patient Discharge Instructions**

You have been admitted with sickle cell crisis,
 You had a prolonged hospitalization, the main issue has been your pain control,
 You have gradually been transition you from subcu to oral hydromorphone.
 You will be discharged home on hydromorphone CR and short acting hydromorphone as well as Tylenol, Celebrex, Voltaren topical cr
 Your were seen by social worker, provided with support, and have been referred to TAMPI pain clinic, NYGH mental health program for mindfulness for her medical condition, and referred to North York, community Settlement worker. You have also have been provided with info re social/mental health support related to sickle cell in the community.
 Awaiting to get appointment for RBC clinic and time for OR re AVC with Dr. Mehdian

Discharge Summary

KIKADI, Ms. PATRICIA KAJINGA - 2519410

Result type: Discharge Summary
Service date: 2026-May-09 15:05 EDT
Result status: Auth (Verified)
Result title: Discharge Summary
Performed by: Mollanji, Roland MD, CCFP, FCFP on 2026-May-09 15:06 EDT
Verified by: Mollanji, Roland MD, CCFP, FCFP on 2026-May-09 15:06 EDT
Encounter info: 0009545548, NYGH, Inpatient, 2026-Apr-01 -

FAX TO UHN RBC CLINIC
416 340 4559

Name: KIKADI, PATRICIA KAJINGA
Address: 2911 BAYVIEW AVENUE
210A
NORTH YORK, ONTARIO M2K1E8
Sex: Female
Date of Birth: 06/19/1987
Phone: 4376635550
Emergency Contact: KIKADI, DEDE
Age: 38 Years

MRN: 2519410
Location: North York General Hospital
Health Card Number: 4557552181 KJ
Family Physician: Ibiyoye, Ademola
Babatunde

Visit Information

Admit Date/Time: 01-APR-2026 12:19
Encounter Type: Inpatient
Attending Provider: Mollanji, Roland MD,
CCFP, FCFP

Hospital Course

ID: 38 F, with history of Sickle Cell, readmitted with vaso occlusive crisis five days after discharge for the same problem.

PMedhx:

1. Sickle cell disease (S/Beta thalassemia)
 - Recently referred to the UHN RBC clinic but has not yet had an appointment
 - AVN of right hip, referred by family physician to orthopedic surgery but was still waiting for appointment
 - Previous documentation reports hemoglobin SS disease, however hemoglobin electrophoresis shows compound heterozygote of hemoglobin S and beta thalassemia
- ay NYGH Nov 10 2025

HPI:

Ms. Kikadi was recently admitted to North York General Hospital from February 24 - March 26 to vaso-occlusive crisis. She was discharged on fentanyl patch and long-acting hydromorphone which were not effective. She was readmitted April 1. She had PICC line inserted on presentation due to poor IV access. She was recently seen in preop clinic March 30 with planned transfusion on April 7, prior to hip replacement on April 15 for AVN. She immigrated to Canada in October 2025 and has spent majority of her time in hospital with vaso-occlusive crisis, one of her triggers is cold weather. She successfully underwent the right hip replacement on April 15.

Recently she she has been stable and we have continued to work on her pain pain. She had some chest pain, but it likely MSK related. CTPA was negative for PE. She has improved and she will be discharged home today.

1. Vaso-occlusive crisis- stable

- Pain in joints, AVN hip is a significant source of pain
- CXR clear
- She has been treated with HM 0.5mg 0.75 q2hrs PRN and prn oral HM 1-3mg which was changed to standing.
- Increased fentanyl patch to 50mcg and go up further as needed as well as we go down on IV HM.

Anticipated Discharge Date/Time and Disposition

Discharge Patient - Ordered
-- 05/09/26 20:00:00 EDT, Discharging
to: Home, if medically stable

Procedure(s)

- Hip Arthroplasty Anterior Biomet
Uncem Taperloc (Right)
(04/15/2026)

Allergies

tramadol Itch, Dizziness, Vomiting

Last Charted Vitals & Measurements (Last 24 Hrs)

T: 36.3 °C (Oral) TMIN: 36.3 °C (Oral)
TMAX: 36.9 °C (Oral)
HR: 85 (Peripheral) RR: 18 BP: 115/60
SpO2: 98%

Selected Lab Results (Last 24 Hrs)

No qualifying data available.

Discharge Summary

KIKADI, Ms. PATRICIA KAJINGA - 2519410

Result type: Discharge Summary
 Service date: 2026-May-09 15:05 EDT
 Result status: Auth (Verified)
 Result title: Discharge Summary
 Performed by: Mollanji, Roland MD, CCFP, FCFP on 2026-May-09 15:06 EDT
 Verified by: Mollanji, Roland MD, CCFP, FCFP on 2026-May-09 15:06 EDT
 Encounter info: 0009545548, NYGH, Inpatient, 2026-Apr-01 -

	What	How Much	When	Instructions
<i>Unchanged</i>	cyanocobalamin (Vitamin B12)	1,000 Microgram Oral	Daily	
<i>Unchanged</i>	diclofenac topical (VOLTAREN 1.16% EMULGEL)	1 application Topical	3 times daily as needed for shoulder pain	Printed Prescription
<i>Unchanged</i>	folic acid	5 Milligram Oral	Daily	Printed Prescription
<i>Unchanged</i>	hydroxyurea	1,000 Milligram Oral	Daily	Printed Prescription
<i>Unchanged</i>	pantoprazole	40 Milligram Oral	Before breakfast	Printed Prescription
<i>Unchanged</i>	pregabalin	75 Milligram Oral	Twice daily	Printed Prescription
<i>Unchanged</i>	senna (Senokot)	2 tab(s) Oral	At bedtime as needed for Constipation	

INFORMATION FOR PATIENT:**Patient Discharge Instructions**

Hip Right, Reason: Follow up Post-Operative, Requested for: 05/19/26 13:50:00 EDT, Priority: Routine, Transport Mode: Stretcher

You have been admitted with sickle cell crisis and you had R Hip replacement on April 15 -Your pain has improved. You will need follow up with Dr. Mehdian at Fracture clinic on May 19 at 13:50.

Take medication as prescribed. Seek medical attention if you have any fever, chills, chest pain, shortness of breath, severe pain, diarrhea or new acute symptoms.

Follow up with RBC clinic at UHN and follow up with your FD within one week.

Scheduled Appointments

Appointment Type	When	Where	Contact Information	Status
FC Fracture New Patient TBS Order	05/19/2026 01:50 PM EDT	Fracture Clinic AMB	416-756-6970	Confirmed

Signature Line

Electronically Signed By: Mollanji, Roland MD, CCFP, FCFP

On: 09-May-2026 15:06

Copies To:

Ibiyooye, Ademola Babatunde

Mehdian, Hossein MD, FRCSC

The family physician on record will automatically receive a copy of this report.

PLEASE NOTE: Elements of this report were generated using electronic technology including speech recognition.

We're the same team with a new name, providing the highest standard of accredited patient care!

REQUEST FOR EXAMINATION – GTA GENERAL SERVICES

- | | | | | | |
|--|---|---|------------------------------------|--------------------------------------|------------------------------------|
| ☐ Brampton Centre | ☐ Brampton Chrysler | ☐ Brampton Sleep Disorders | ☐ Milton | ☐ Mississauga | ☐ Newmarket |
| ☐ North York | ☐ Orangeville | ☐ Oshawa | ☐ Pickering | ☐ Scarborough | ☐ Thornhill |
| ☐ Toronto Bay | ☐ Toronto Davisville | ☐ Toronto King | ☐ Whitby | | |

PATIENT INFORMATION (AFFIX LABEL IF AVAILABLE)

Check if Applicable: ☐ **URGENT**

Full Name (Birth): Patricia Kikadi

Preferred Full Name (if Different from Birth): _____

Address: A210-2911 Bayview Ave

City: North York Prov.: ON Postal Code: _____

Cell Phone: (437) 663-5550 Alt. Phone: (437) 663-5550

Date of Birth: 1987-Jul-19 Health Card #: 4557552181KJ Version: _____

Gender (Birth): F Preferred Gender (if Different from Birth): _____

Height (cm): _____ Weight (kg): _____

Reason for Referral:

Bilateral hip pain with hx of HBSS r/o cause restriction of movt of LT shoulder

ULTRASOUND

GENERAL ULTRASOUND

- ☐ Abdomen + Pelvis (incl. reproductive organs)
- ☐ Abdomen (incl. limited bladder + lower quadrants, no reproductive organs)
- ☐ Kidneys*
- ☐ Bladder
- ☐ Hernia (specify site): _____
- ☐ Other: _____

*Baseline abdominal ultrasound may be performed

PELVIS

- ☐ Female Pelvis (incl. Transvaginal)
- ☐ Male Pelvis (Excl. Transrectal)

OBSTETRICAL

- EDC (Required):** _____
- ☐ Dating (< 16 weeks)
- ☐ Prenatal Screening (IPS/eFTS 11-14 weeks)
- ☐ Anatomy (18-20 weeks)
- ☐ Dual Scan Series (NT scan 11-14 weeks + Anatomical 18-20 weeks)
- ☐ Fetal Growth (30+ weeks)
- ☐ BPP ☐ UA Doppler ☐ MCA Doppler
- ☐ Biophysical Profile (BPP)
- ☐ Twin Series (> 18 weeks) Site Specific
- ☐ Follicular Study

US GUIDED PROCEDURES

- ☐ Biopsy – Thyroid FNA Site Specific
- ☐ Biopsy – Breast Site Specific
- ☐ Sonohysterogram Site Specific

MUSCULOSKELETAL

- R L**
- ☒ Shoulder
- ☐ Elbow
- ☐ Wrist
- ☐ Hip
- ☐ Hamstring
- ☐ Knee
- ☐ Ankle/Achilles Tendon/Plantar Fascia (circle one above)
- ☐ Other: _____

SMALL PARTS

- ☐ Salivary Glands
- ☐ Thyroid
- ☐ Chest
- ☐ Groin ☐ R ☐ L
- ☐ Inguinal Canal ☐ R ☐ L
- ☐ Testes/Scrotum
- ☐ Soft Tissue/Lump (specify site): _____

VASCULAR

- R L**
- ☐ Venous - Lower Extremity (DVT)
- ☐ Venous - Upper Extremity (DVT)
- ☐ Venous - Lower Extremity (Reflux)
- ☐ Arterial - Lower Extremity (ABI)
- ☐ Arterial - Upper Extremity
- ☐ Carotid
- ☐ Renal Arteries
- ☐ Portal Venous Doppler
- ☐ Aorta:
- ☐ OTHER: _____

SPECIALIST CONSULTATIONS

- ☐ First Available: ☐ Cardiologist ☐ Internist ☐ Sleep Medicine
- ☐ Dr. _____ ☐ Consult if Test Result is Positive/Abnormal
- Please Attach: Medications, Previous Tests, Family & Social History

CARDIOLOGY

- | | |
|---|--|
| ☐ 12-Lead Electrocardiogram (Rest ECG) | ☐ Stress Echocardiogram |
| ☐ Exercise Stress Test (GXT) | ☐ Echocardiogram (Colour Doppler) |
| ☐ Holter Monitoring | ☐ Contrast Echocardiogram |
| ☐ 24 hrs ☐ 48 hrs ☐ 72 hrs | ☐ Chest pain suspicious of CAD |
| ☐ Other: _____ | ☐ CHF ☐ Palpitations/Arrhythmias |
| ☐ 24hr BP Monitor (Not insured by OHIP) | ☐ Hypertension ☐ Murmur ☐ Syncope |
| ☐ Pulmonary Function Testing (PFT) | ☐ Other: _____ |
| ☐ Pre & Post Spirometry | |
| ☐ Full Pulmonary Function Test | |
| ☐ Include Respiriology Consult | |

SLEEP DISORDERS

- ☐ Consultation & Sleep Study
- ☐ Consultation Only ☐ Sleep Study Only

NUCLEAR CARDIOLOGY

- | | |
|---|------------------------------------|
| MYOCARDIAL PERFUSION | VENTRICULAR FUNCTION |
| ☐ Exercise ☐ Persantine ☐ Dobutamine | ☐ Rest MUGA |

X-RAY (WALK-IN SERVICE)

- | | | |
|---|---|--|
| ABDOMINAL | LOWER EXTREMITIES | UPPER EXTREMITIES |
| ☐ Single/KUB | R L | R L |
| ☐ Acute (incl. PA chest) | ☒ Hip | ☒ Shoulder |
| CHEST | ☐ Femur | ☐ Clavicle |
| ☐ Chest PA & LAT | ☐ Knee | ☐ Sternoclavicular Joints |
| ☐ Ribs ☐ R ☐ L | ☐ Tib. & Fib. | ☐ A.C. Joint |
| ☐ Sternum | ☐ Ankle | ☐ Scapula |
| ☐ Chest Visa | ☐ Foot | ☐ Humerus |
| HEAD & NECK | ☐ Calcaneus | ☐ Elbow |
| ☐ Soft Tissue Neck | ☐ Toe: | ☐ Forearm |
| ☐ Skull | SPINE & PELVIS | ☐ Wrist |
| ☐ Sinuses (Not insured by OHIP) | ☐ Cervical Spine | ☐ Scaphoid |
| ☐ Facial Bones | ☐ Thoracic Spine | ☐ Hand |
| ☐ Nose | ☐ Lumbar (L/S) Spine | ☐ Finger: |
| ☐ Mandible | ☐ Sacrum/Coccyx | |
| ☐ Orbits | ☐ S.I. Joints | OTHER |
| ☐ T.M. Joints | ☒ Pelvis | ☐ Skeletal Survey |
| ☐ Adenoids | ☐ Scoliosis Series | ☐ Bone Age |
| | | ☐ Indicate: _____ |



MAMMOGRAPHY & WOMEN'S IMAGING

- ☐ Targeted Breast Ultrasound* (Indicate quadrant on diagram) ☐ R ☐ L
- ☐ Mammogram ☐ R ☐ L ☐ Implants
- ☐ Mammogram & Bone Mineral Density
- ☐ R ☐ L ☐ Implants | ☐ Baseline ☐ Follow Up



*Breast ultrasound is not used for screening purposes. Mammogram/OBSP is recommended.

BONE MINERAL DENSITY

- ☐ Baseline ☐ Follow Up

NUCLEAR MEDICINE

- | | |
|---|--|
| BONE SCAN | RENAL |
| ☐ Total Body ☐ Specific Site: _____ | ☐ Renal Scan with Differential Function |
| ENDOCRINE | MISCELLANEOUS |
| ☐ Thyroid Uptake & Scan | ☐ V/Q Lung Scan |
| ☐ Parathyroid | PET/CT – Mississauga |
| GASTROINTESTINAL | ☐ Visit WELldiagnostics.ca/Refer for PET/CT requisition |
| ☐ Hepatobiliary Scan (HIDA) | |
| ☐ Solid Gastric Emptying Scan | |

REFERRING HEALTHCARE PROVIDER (STAMP LABEL IF AVAILABLE)

Referring Provider: Dr. Ademola Babatunde biyooye (Print Name) [Signature] (Signature)

Billing Provider #: _____ CPO # 046709

Tel #: (289) 825-8044 Fax #: (289) 825-8043

Date: 2025-Oct-25 Copy To: _____

Report Delivery Preference: ☐ Fax ☐ HRM ☐ Other: _____

Access your patient radiology reports at WELldiagnostics.ca/Access



Ministry of Health
and Long-Term Care
Laboratory Requisition
Requisitioning Clinician / Practitioner

Dr. Ademola Babatunde Ibiyoye

Address

2760 Derry Road West, Unit 7
Mississauga, ON
L5N 3N5

Clinician/Practitioner Number

CPSO / Registration No.

046709

129492

Check (✓) one:

☒ OHIP/Insured

☐ Third Party / Uninsured

☐ WSIB

Additional Clinical Information (e.g. diagnosis)

☐ Copy to: Clinician/Practitioner

Last Name

First Name

Address

Laboratory Use Only

Clinician/Practitioner's Contact Number for Urgent Results
(289) 825-8044

Service Date
yyyy mm dd

Health Number

Version

Sex

4557552181

KJ

F

Date of Birth
yyyy mm dd
1987 06 19

Province Other Provincial Registration Number

ON

Patient's Telephone Contact Number
(H): (437) 663-5550
(C): (437) 663-5550

Patient's Last Name (as per OHIP Card)

Kikadi

Patient's First & Middle Names (as per OHIP Card)

Patricia

Kajinga

Patient's Address (including Postal Code)

A210-2911 Bayview Ave
North York, ON
M2K 1E8

Note: Separate requisitions are required for cytology, histology / pathology, ColonCancerCheck FIT test, and tests performed by Public Health Laboratory

x Biochemistry		x Hematology		x Viral Hepatitis (check one only)	
☐ Glucose	☐ Random ☐ Fasting	☒ CBC		☐ Acute Hepatitis	
☐ HbA1C		☒ Prothrombin Time (INR)		☒ Chronic Hepatitis	
☐ Creatinine (eGFR)		Immunology		☐ Immune Status / Previous Exposure	
☐ Uric Acid		☐ Pregnancy Test (Urine)		Specify: ☐ Hepatitis A	
☐ Sodium		☐ Mononucleosis Screen		☐ Hepatitis B	
☐ Potassium		☐ Rubella		☐ Hepatitis C	
☒ ALT		☐ Prenatal: ABO, RhD, Antibody Screen (titre and ident. if positive)		or order individual hepatitis tests in the "Other Tests" section below	
☒ Alk. Phosphatase		☐ Repeat Prenatal Antibodies		Prostate Specific Antigen (PSA)	
☒ Bilirubin		Microbiology ID & Sensitivities (if warranted)		☐ Total PSA	☐ Free PSA
☒ Albumin		☐ Cervical		Specify one below:	
☐ Lipid Assessment (includes Cholesterol, HDL-C, Triglycerides, calculated LDL-C & Cholesterol/HDL-C ratio; individual lipid tests may be ordered in the "Other Tests" section of this form)		☐ Vaginal		☐ Insured - Meets OHIP eligibility criteria	
☐ Albumin / Creatinine Ratio: Urine		☐ Vaginal / Rectal - Group B Strep		☐ Uninsured - Screening: Patient responsible for payment	
☐ Urinalysis (Chemical)		☐ Chlamydia (specify source):		Vitamin D (25-Hydroxy)	
☐ Neonatal Bilirubin		☐ GC (specify source):		☐ Insured - Meets OHIP eligibility criteria	
Child's Age: _____ days _____ hours		☐ Sputum		osteopenia; osteoporosis; rickets; renal disease; malabsorption syndromes; medications affecting vitamin D metabolism	
Clinician/Practitioner's tel. no. _____		☐ Throat		☐ Uninsured - Patient responsible for payment	
Patient's 24 hr telephone no. _____		☐ Wound (specify source):		Other Tests - one test per line	
☐ Therapeutic Drug Monitoring		☐ Urine		GGT	
Name of Drug #1 _____		☐ Stool Culture		ceruloplasmin	
Name of Drug #2 _____		☐ Stool Ova & Parasites		ANA	
Time Collected #1 _____ hr #2 _____ hr		☐ Other Swabs / Pus (specify source):		A1AT	
Time of Last Dose #1 _____ hr #2 _____ hr				Serum iron	
Time of Next Dose #1 _____ hr #2 _____ hr				TIBC	
				transferrin	

I hereby certify the tests ordered are not for registered in or out patients of a hospital.

Specimen Collection

Time

Date

YYYY/MM/DD

Laboratory Use Only

X

Clinician/Practitioner Signature

2025/11/08

Date



Ministry of Health
and Long-Term Care
Laboratory Requisition
Requisitioning Clinician / Practitioner

Dr. Ademola Babatunde Ibiyoye

2760 Derry Road West, Unit 7
Mississauga, ON
L5N 3N5

Clinician/Practitioner Number

046709

CPSO / Registration No.

129492

Check (✓) one:

☒ OHIP/Insured

☐ Third Party / Uninsured

☐ WSIB

Additional Clinical Information (e.g. diagnosis)

☐ Copy to: Clinician/Practitioner

Last Name

First Name

Address

Laboratory Use Only

Clinician/Practitioner's Contact Number for Urgent Results
(289) 825-8044

Service Date
yyyy mm dd

Health Number

4557552181

Version

KJ

Sex

F

Date of Birth

yyyy

1987

mm

07

dd

19

Province Other Provincial Registration Number

ON

Patient's Telephone Contact Number

(H): (437) 663-5550

(C): (437) 663-5550

Patient's Last Name (as per OHIP Card)

Kikadi

Patient's First & Middle Names (as per OHIP Card)

Patricia

Kajinga

Patient's Address (including Postal Code)

A210-2911 Bayview Ave

North York, ON

M2K 1E8

Note: Separate requisitions are required for cytology, histology / pathology, ColonCancerCheck FIT test, and tests performed by Public Health Laboratory

x Biochemistry		x Hematology		x Viral Hepatitis (check one only)	
☒ Glucose	☒ Random ☐ Fasting	☒ CBC		☐ Acute Hepatitis	
☒ HbA1C		☐ Prothrombin Time (INR)		☐ Chronic Hepatitis	
☒ Creatinine (eGFR)		Immunology		☐ Immune Status / Previous Exposure	
☒ Uric Acid		☐ Pregnancy Test (Urine)		Specify ☐ Hepatitis A	
☒ Sodium		☐ Mononucleosis Screen		☐ Hepatitis B	
☒ Potassium		☐ Rubella		☐ Hepatitis C	
☒ ALT		☐ Prenatal ABO RhD Antibody Screen (titre and ident. if positive)		or order individual hepatitis tests in the "Other Tests" section below	
☒ Alk. Phosphatase		☐ Repeat Prenatal Antibodies		Prostate Specific Antigen (PSA)	
☒ Bilirubin		Microbiology ID & Sensitivities (if warranted)		☐ Total PSA	☐ Free PSA
☒ Albumin		☐ Cervical		Specify one below:	
☒ Lipid Assessment (includes Cholesterol, HDL-C, Triglycerides, calculated LDL-C & Chol/HDL-C ratio; individual lipid tests may be ordered in the "Other Tests" section of this form)		☐ Vaginal		☐ Insured - Meets OHIP eligibility criteria	
☒ Albumin / Creatinine Ratio, Urine		☐ Vaginal / Rectal - Group B Strep		☐ Uninsured - Screening, Patient responsible for payment	
☒ Urinalysis (Chemical)		☐ Chlamydia (specify source)		Vitamin D (25-Hydroxy)	
☐ Neonatal Bilirubin		☐ GC (specify source)		☐ Insured - Meets OHIP eligibility criteria	
Child's Age days hours		☐ Sputum		osteopenia, osteoporosis, rickets; renal disease; malabsorption syndromes; medications affecting vitamin D metabolism	
Clinician/Practitioner's tel. no.		☐ Throat		☐ Uninsured - Patient responsible for payment	
Patient's 24 hr telephone no.		☐ Wound (specify source)		Other Tests - one test per line	
☐ Therapeutic Drug Monitoring		☐ Urine		TSH	
Name of Drug #1		☐ Stool Culture		Vit B12	
Name of Drug #2		☐ Stool Ova & Parasites		Ferritin	
Time Collected #1 hr. #2 hr.		☐ Other Swabs / Pus (specify source)		CRP	
Time of Last Dose #1 hr. #2 hr.					
Time of Next Dose #1 hr. #2 hr.					

I hereby certify the tests ordered are not for registered in or out patients of a hospital.

Clinician/Practitioner Signature

2025/10/25

Date

Specimen Collection

Time

Date

YYYY/MM/DD

Laboratory Use Only

DR. HOSSEIN MEHDIAN MEDICINE PROFESSIONAL CORPORATION

Dr. H. Mehdian
Orthopaedic Surgeon

800-240 Duncan Mill Rd.
Toronto, Ontario M3B 3S6
Tel: 416-850-4064
Fax: 416-850-6681

JOINT REPLACEMENT **TEXT LINE: 437 249 3808**

NAME: PATRICIA KIKADI

DATE OF SURGERY: WED APR 15/26 @ 12:45 PM- **ARRIVE BY 10:30 AM**

DATE OF PRE OP APPT: MAR 30/26 @ 7 AM

Your surgery and pre op will take place at: North York General Hospital
4001 Leslie St.
Toronto, ON
416-756-6000

PRE OP APPT INSTRUCTIONS

- The Anesthetic Questionnaire you have received should be filled out by you and brought with you to the pre-op appointment.
- Register at Patient registration
- Go to the PRE OP clinic located on the 4th floor.
- If you are unable to make it to the appointment, please call and reschedule @ 416-756-6375

SURGERY INSTRUCTIONS

- You must call 416-850-4064 or text 437 249 3808 a week before your surgery to confirm the time of your surgery.
- PLEASE DO follow any instructions given @ pre op.
- Please bring 2 of your own **REUSABLE BAGS** on the day of your procedure. One big enough for your shoes and one big enough for your clothing. Reusable cup to use after surgery.

You are required to watch the "Total Joint Replacement Surgery at NYGH" video prior to your PRE OP APPT. It can be found at:

www.nygh.on.ca/hip-knee-education under resources and videos

PLEASE BE AWARE THAT YOUR SURGERY DATE IS SUBJECT TO CHANGE ON SHORT NOICE DUE TO SURGICAL EMERGENCIES.

If you have any further questions, please call the office.

Rx Physician's Prescription
GTA Post Surgery Care

To Rent or Purchase a device or learn more:

Call: 1-877-830-3167 or 519-712-1612 or

Email: gtapostopcare@gmail.com

Date of Surgery: Apr 15/26

☒ Surgical

☐ Non-Surgical

Patients Name: Patricia Kikadi

Email: _____

Address: _____

Alt Contact: _____

Phone #: 613 869 4129

Alt Phone: _____

I am prescribing a pneumatic Intermittent Compression and Cold Therapy System (Breg and/or Game Ready) due to my patient's needs and diagnosis. I certify that both the BREG and Game Ready device is medically indicated and in my opinion is reasonable and necessary with reference to the accepted standards of medical practice and treatment of this patient's condition

Product: **Game Ready Control Unit Rental**

Rental Duration: ☐ 2 to 4 weeks ☐ 4 to 8 weeks

or

Breg Polar Care Wave Purchase - Complete with wrap

WRAP:

☐ Ankle ☐ Articulated Knee ☐ Back ☐ C-T Spine ☐ Elbow
☐ Half-Leg Boot ☐ Hand/Wrist ☒ Hip/Groin ☐ Knee ☐ Shoulder

SIDE:

☐ Left ☒ Right

Torso or Shoulder Extender: ☐ 8 inches or 10 inches

Rx Physician's Letter of Medical Necessity

I am writing on behalf of my patient that you approve coverage for the BREG or Game Ready vasopneumatic compression and cold therapy system. I consider this device medically necessary, and I am prescribing this device for the purpose of musculoskeletal injury treatment and/or post-operative treatment.

Both systems combine cold and compression therapies. It is intended to treat post-surgical and acute injuries to reduce edema, swelling and pain where cold and compression are indicated.

RICE (Rest, Ice, Compression, and Elevation) has long been used to treat acute and chronic injury and assist in rehabilitation following orthopedic surgery. BREG and/or Game Ready combines the two most difficult to manage aspects of the RICE regimen (Ice and Compression) by offering adjustable cold and intermittent compression in one easy-to-use system. The anatomically-designed wraps are engineered for all major body parts, and utilize intermittent pneumatic compression and fluid circulation technology, simultaneously delivering circumferential cold and compression to most major joints.

My post-operative and rehabilitative care plan calls for the use of the BREG or Game Ready device to reduce pain and swelling. Failure to control pain not only causes unnecessary suffering, but can delay my patient's recovery. Therefore, the need for compliance with the required treatment is high. I certify that the above-described products are medically indicated and in my opinion are reasonable and necessary. Given the safety and effectiveness of this unit, I prescribe and recommend that the patient use this device daily. Without use of this device, there is potential to cause unnecessary delay in the patient's recovery.

Physician Signature: Dr. M. Kikadi

Date: Nov 7/26

Physicians Printed Name: Dr. M. Kikadi

CPSO#

73227



**NORTH
YORK
GENERAL**
*Making a World
of Difference*

Anterior Approach
Hip Replacement Exercises
To be done 3 times per day

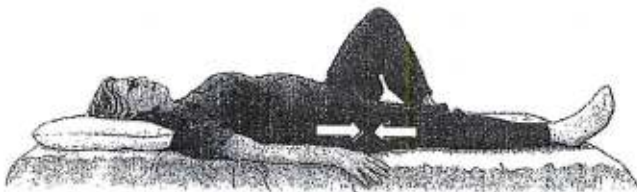
Patricia
May 7, 2026

1. Move feet up and down at the ankles.



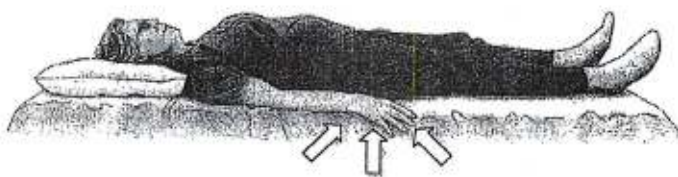
Repeat 30 times

2. With operated leg straight press down the back of your knee(s) into the bed, tighten the muscles on the front of your thigh.



Hold for 5 seconds, Repeat 10 times

3. Lying on your back. Squeeze your buttock muscles together.



Hold for 5 seconds, Repeat 10 times

4. Bend your knee on the operated side, sliding the heel of your foot up the bed toward your buttocks.



Repeat 10 times

Gulshan and Pyarali G. Nanji
Orthopaedic & Plastic Centre

4001 Leslie Street
 Toronto, Ontario M2K 1E1
 416 756-6970 main
 416 756-6502 fax
www.nygh.on.ca



**NORTH
 YORK
 GENERAL**

*Making a World
 of Difference*

MRN = 2519410 FIN = 9619762
KIKADI, PATRICIA KAJINGI

1987-JUN-19 F 38 Years
 210A-2911 BAYVIEW AVENUE NORTH YORK M2K1E8
 (437) 663-5550 2026-JUN-85
 (FM) 181Y00YE, ADEMO (AT) MEHDIAN, HOSSEI
 Fracture ARM

4557552181-KJ



☐ **Outpatient Physiotherapy**

☐ **HIP AND KNEE CLASS**

North York General Hospital
 5 South Room 172
 416 756-6000 ext. 4784

☐ **Dr. Herman Dhotar**
 243-1333 Sheppard Ave. E.
 Toronto, ON M2J 1V1
 416 492-5633 416 495-1590 fax

☐ **Dr. Shawn Garbedian**
 226-1333 Sheppard Ave. E.
 Toronto, ON M2J 1V1
 416 850-4064 416 850-6681 fax

☐ **Dr. Robert Brock**
 225-1333 Sheppard Ave. E.
 Willowdale, ON M2J 1V1
 416 494-7431 416 494-7796 fax

☐ **Dr. Gladys Chan**
 243-1333 Sheppard Ave. E.
 Willowdale, ON M2J 1V1
 416 494-7351 416 494-7446 fax

☐ **Dr. Dale Dantzer**
 1333 Sheppard Ave. E., Suite 243
 Toronto, ON M2J 1V1
 416 492-5633 416 495-1590 fax

☐ **Dr. Ian Mayne**
 225-1333 Sheppard Ave. E.
 Toronto, ON M2J 1V1
 647 340-6400 416 907-1777 fax

☐ **Dr. Hossein Mehdian**
 226-1333 Sheppard Ave. E.
 Willowdale, ON M2J 1V1
 416 850-4064 416 850-6681 fax

☐ **Dr. Christopher Peskun**
 225-1333 Sheppard Ave. E.
 Willowdale, ON M2J 1V1
 647 340-6400 416 494-7796 fax

☐ **Dr. Frank Mastrogiacomo**
 658 Danforth Ave.
 Toronto, ON M4K 1R2
 416 406-5666 416 406-6667 fax

☐ **Dr. Arun Sayal (Emerg MD)**
 Orthopaedic & Plastic Centre
 4001 Leslie Street
 Toronto, ON M2K 1E1

☐ **Dr. Yasser S. El Sheikh**
 208-1333 Sheppard Ave. E.
 Toronto, ON M2J 1V1
 416 782-7788 416 491-7022 fax

☐ **Dr. Mary-Helen Mahoney**
 1333 Sheppard Ave. E.
 Toronto, ON M2J 1V1
 416 554-9860 416 491-7022 fax

☐ **Dr. Sean Rice**
 345-1333 Sheppard Ave. E.
 Toronto, ON M2J 1V1
 416 391-4048 416 391-3979 fax

☐ **Dr. Petra Schiller**
 406-1100 Sheppard Ave. E.
 Willowdale, ON M2K 2W1
 416 492-3613 416 492-0635 fax

☐ **Dr. Ron Somogyi**
 316-20 Wynford Dr.
 Toronto, ON M3C 1J4
 416 447-6176 416 447-5750 fax

☐ **Dr. Waqqas Jalil**
 316-20 Wynford Dr.
 Toronto, ON M3C 1J4
 416 855-8149 416 447-5750 fax

Date: June 5, 2020

Referred for: Post-Op Physiotherapy ☐ Physiotherapy ☒
 Work Conditioning ☐ Strength/Conditioning Rehab ☐ At discretion of PT ☐

Current Primary Diagnosis: Left Shoulder

Surgical Procedure Performed: AVN w screw

Date of Injury/Surgery: 2 1/2 yrs. ago. fell.

Precautions: No.

Weight Bearing Status: _____

Relevant Clinical Findings: ROM / Strength

Rehabilitation Goals: Local mobility.

Referring Physician's Signature H. Mehdian

Please note the following requirements:

1. Written reports to be given to the patient for their follow-up appointment with the Surgeon.
2. Discharge Summary-faxed/mail to the Doctor.

73227